

4 discrete product groups

1. Pure platform (Common feature set and intended use that is not explicit to the functionality of the regulated software components)
2. Controlled (Has the potential to be considered SaMD, strict limits and controls on how this software performs is required to maintain non-MD status)
3. Regulated SaMD
4. AI-specific layer (future-proofing for EU AI Act)

GROUP 1 — Core Platform (Non-Device)

Intended Purpose

A software platform for the secure collection, storage, organisation, communication, and presentation of patient data and healthcare workflow information. The system does not perform clinical interpretation, prioritisation, or provide information for diagnosis, monitoring, or treatment decisions.

Includes (from your system)

- Patient management
- Submissions capture
- Media storage & viewing
- Messaging & communication
- Worklists (manual only)
- Dashboards (raw data only)
- Integrations
- Audit, auth, infrastructure

Explicit exclusions

- No scoring
- No flags/alerts
- No prioritisation logic
- No AI interpretation

Regulatory position

- **NOT a medical device**
- Outside UK MDR

GROUP 2 — Controlled Clinical Tools (Non-Device)

Intended Purpose

Software functions that transform, organise, or summarise patient data to support clinical workflow efficiency without performing clinical interpretation or providing recommendations for diagnosis or treatment.

G2 functionality is non-medical by design and must not introduce interpretation, prioritisation, or decision-support language.

Includes

- Timeline views
- Submission review aggregation
- Trends (**visual only**)
- Image quality checks (blur detection)
- Worklist filtering/sorting (non-clinical)
- Pathways (non-clinical logic only)

Strict controls

- No language implying clinical significance
- No prioritisation based on risk
- No “insight”, “alert”, “concern”, etc.
- No automated decision branching based on clinical thresholds
 - These features drift into SaMD
 - Or contaminate your Core Platform

Regulatory position

- **NOT a medical device (by design control)**
- Requires **internal regulatory governance**

GROUP 3 — Clinical Decision Support (SaMD)

Intended Purpose

Software intended to analyse, process, and/or interpret patient data to provide information that supports clinical decision-making for the diagnosis, monitoring, or management of disease or medical conditions.

Includes

- Form scoring
- Flags/alerts
- Decision nodes
- Clinical rules engines / automated actions
- Interpreted trends (if used clinically)
- AI-generated clinical summaries (Locked down)

Regulatory classification

- **Medical Device (SaMD)**
- Full risk requirement
- Classification dependent on risk and benefit, controls in place as a medical device

Key requirement

- Must be **functionally and technically separable** from Groups 1 & 2

GROUP 4 — AI / ML Engine (High-Risk AI Layer)

This separates **AI-specific obligations under the EU AI Act**.

Intended Purpose

TBC.

Includes

- SSI detection
- ML feature detection
- OCR (if clinically interpreted)
- Future AI models

Regulatory position

- If used clinically:
 - **SaMD**
 - **High-Risk AI (EU AI Act)**

Why separate from Group 3

- AI Act introduces:
 - Data governance requirements
 - Model lifecycle controls
 - Monitoring obligations

Appendix I - Feature Mapping Table

Level 1

Feature	Cohort	Group	Can Escalate to	Trigger for Escalation	Regulatory Rationale	Risk Type	Key Controls
Patient management	Data Management	G1	G3	Introduction of clinical interpretation or decision logic	Storage and organisation of data only	N/A	No interpretation permitted
Messaging	Communication	G1	G3	Addition of clinical decision support or automated triage	Communication only, no clinical function	N/A	No clinical logic or automation
Media storage & viewer	Data Management	G1	G3	Interpretation or analysis of media content	Passive display of data	N/A	No transformation of meaning

Worklists (manual)	Workflow	G1	G3	Automated prioritisation or risk-based sorting	User-driven workflow only	N/A	No automated logic
Dashboards (raw data)	Workflow	G1	G3	Addition of interpretation, scoring, or alerts	Display only	N/A	No analytics layer
Integrations (EPR, APIs)	Infrastructure	G1	G3	Data transformation influencing clinical meaning	Data transfer only	N/A	Data integrity controls
Audit / authentication	Infrastructure	G1	N/A	N/A	System-level functionality only	N/A	Security controls

Level 2

Feature	Cohort	Group	Can Escalate to	Trigger for Escalation	Regulatory Rationale	Risk Type	Key Controls
Timeline view	Aggregation	G2	G3	Highlighting deterioration or clinical significance	Organisation of data without interpretation	Misinterpretation	No clinical labels or flags
Submission aggregation	Aggregation	G2	G3	Deriving clinical meaning from grouped data	Grouping for convenience only	Misinterpretation	No transformation of meaning
Trends (visual only)	Visualisation	G2	G3	Interpretation (e.g. “worsening”, thresholds)	Visual display only	Misleading presentation	No thresholds or alerts
Submission dashboard	Visualisation	G2	G3	Prioritisation, scoring, or alerts	Display of data only	Misinterpretation	No ranking or scoring
Worklist filtering/sorting	Workflow	G2	G3	Risk-based or automated prioritisation	Operational efficiency only	Misprioritisation	No clinical sorting logic
Pathway builder	Workflow	G2	G3	Branching based on patient condition	Static workflow only	Incorrect pathway use	No clinical decision branching
Automated actions builder	Workflow	G2	G3	Triggered by clinical thresholds or patient state	Operational automation only	Unintended decision support	No clinical triggers
OCR (data extraction only)	Processing	G2	G3	Output used for clinical interpretation	Data extraction only	Data accuracy	No interpretation of extracted data
Image quality checks (blur detection)	Quality	G2	G3	Framed as clinical usability or adequacy	Technical quality only	Misinterpretation	No clinical claims or framing

Level 3

Feature	Cohort	Group	Can Escalate to	Trigger for Escalation	Regulatory Rationale	Risk Type	Key Controls
Form scoring	Scoring	G3	N/A	N/A	Generates clinically meaningful outputs	Incorrect scoring → harm	Algorithm validation
Risk scoring systems	Scoring	G3	N/A	N/A	Influences clinical decision-making	Misclassification	Clinical evaluation
Alerts / flags	Alerts	G3	N/A	N/A	Directly influences prioritisation	Missed or false alerts	Performance validation
Decision nodes	Decision Logic	G3	N/A	N/A	Applies clinical logic to data	Incorrect decisions	Traceability + verification

Rules engine	Decision Logic	G3	N/A	N/A	Automates clinical rules	Systemic error	Controlled configuration
Interpreted trends	Interpretation	G3	N/A	N/A	Adds clinical meaning to data	Misinterpretation	Clinical validation
AI summaries (clinical use)	Interpretation	G3	N/A	N/A	Synthesises clinical data for decisions	Incorrect summary	Locked, validated models

Level 4

Feature	Cohort	Group	Can Escalate to	Trigger for Escalation	Regulatory Rationale	Risk Type	Key Controls
SSI detection	Detection	G4	N/A	N/A	AI-based clinical interpretation	Missed diagnosis	Model validation
ML feature detection	Detection	G4	N/A	N/A	Automated identification of clinical features	False positives/negatives	Dataset governance
Predictive models	AI Output	G4	N/A	N/A	Predicts outcomes influencing care	Incorrect prediction	Monitoring + PMS
AI-driven insights	AI Output	G4	N/A	N/A	Interprets data beyond raw input	Over-reliance	Explainability controls